FAX

SENSITIVE

DATE:	03/03/2026		
TO:	3033250150	FROM:	3034251378
PAGES	10		

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Western Infectious Disease Consultants PC

1536 Cole Blvd, Suite 120

Lakewood, CO 80401-3405

P: 303-425-9245

F: 303-425-1378

Attention:

Comments:

Lakewood Office Fax: 303-425-1378

Broomfield Office Fax: 720-630-8591

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Fax

To: ELLENBOGEN, CHARLOTTE	**From:** Pam Health at Home - Denver
Fax: 13034251378	**Pages:** 8
Phone:	**Date:** 03-03-2026 8:09 AM
Re: Order(s) for Signature	

Comments:

This fax contains orders for signature.
Please sign, date, and return all pages to 1-303-325-0150.
Thank you!

PAM Health at Home - Denver
9025 E. Kenyon Ave, Suite 100, Denver, CO 80237
Ph: 720-808-0001; Fax: 303-325-0150

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Order No:485-26222

HOME HEALTH CERTIFICATION AND PLAN OF CARE

1. Patient's HI Claim No.	2. Start Of Care Date	3. Certification Period	4. Medical Record No.	5. Provider No.
G076450	02/23/2026	02/23/2026 - 04/23/2026	9704	

6. Patient's Name and Address
VIGIL, JAMES P (9704)
10942 BIRCH DR
THORNTON, CO 80233
Phone: (303) 669-7992

7. Provider's Name, Address and Telephone Number
PAM Health at Home
9025 East Kenyon Ave Ste 50
Denver, CO 80237-1980
Phone: (720) 808-0001, Fax: (720) 279-4371

8. Date of Birth: 04/03/1979 **9. Sex:** ☑ M ☐ F

10. Medications: Dose/Frequency/Route (N)ew (C)hanged

Medication	Dose	Frequency	Route	Status
Oxycodone 5 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	5mg	every 4 hours	oral	LS - PRN/As Needed
Piperacillin-tazobactam 13.5 gram intravenous solution [Amount Of/ Dosage Form: 0 Milliliter]	13.5mg	every 24 hours	intravenous	LS
Simethicone 40 mg/0.6 mL oral drops,suspension [Amount Of/ Dosage Form: 0 Metric Drop]	40mg	every 6 hours	oral	LS - PRN/As Needed
Tylenol Extra Strength 500 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	500mg	every 6 hours	oral	LS - PRN/As Needed

11. ICD-9 / 10 CM Principal Diagnosis

Code	Code Description	Date	O/E
K57.20	Diverticulitis of large intestine with perforation and abscess without bleeding		

12. ICD-9 / 10 CM Surgical Procedure

Code	Code Description	Date	O/E

13. ICD-9 / 10 CM Other Pertinent Diagnosis

Code	Code Description	Date	O/E
L02.211	Cutaneous abscess of abdominal wall		
Z45.2	Encounter for adjustment and management of vascular access device		
N32.1	Vesicointestinal fistula		
Z90.49	Acquired absence of other specified parts of digestive tract		
Z79.2	Long term (current) use of antibiotics		
Z51.81	Encounter for therapeutic drug level monitoring		
Z79.891	Long term (current) use of opiate analgesic		

14. DME and Supplies:

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Supplies:
PICC line supplies

15. Safety Measures:
911 Emergency / Protocol, 911 for Emergency, Assistance with IADLs, CENTRAL LINE PRECAUTIONS, Emergency Plan / Measures, Emergency Plan Developed, Equipment Safety, Fall / Safety Precautions, Fall Precautions, Hazard-Free Environment, Infection Control Standard Measures and Precautions, Keep Pathways Clear, Keep Pathways Clear and Safe, Safe Disposal of Sharp, Contaminated Materials, Safe in Movements, Safe Transfers, Safety in ADLs, Skin Care, Slow Position Change, Standard Precautions/Infection Control, Universal Precautions

16. Nutritional Req.
Soft foods

17. Allergies:
No Known Allergies

18.A. Functional Limitations
☐ 1- Amputation ☐ 4- Hearing ☐ 7- Ambulation
☐ 2- Bowel/Bladder (incontinence) ☐ 5- Paralysis ☐ 8- Speech
☐ 3- Contracture ☑ 6- Endurance ☐ 9 - Legally Blind
☐ A- Dyspnea with minimal exertion
☐ B- Other (Specify):

18.B. Activities Permitted
☐ 1- Complete Bedrest ☑ 5- Exercises Prescribed ☐ 9- Cane
☐ 2- Bedrest BRP ☐ 6- Partial Weight Bearing ☐ A- Wheelchair
☑ 3- Up as Tolerated ☐ 7- Independent at Home ☐ B- Walker
☐ 4- Transfer Bed/Chair ☐ 8- Crutches ☐ C- No Restrictions
☐ D- Other (Specify):

19. Mental Status
☑ 1 - Oriented ☐ 2 - Comatose ☐ 3 - Forgetful ☐ 4 - Depressed ☐ 5 - Disoriented ☐ 6 - Lethargic ☐ 7 - Agitated
☐ 8 - Other(Specify):

20. Prognosis: ☐ 1- Poor ☐ 2- Guarded ☑ 3- Fair ☐ 4- Good ☐ 5- Excellent ☐ 6- Other (Specify):

21. Treatments and Orders for Disciplines:
Frequency
SN Frequency: 2W1, 1W8 Starting Week of 02/23/2026, SN PRN Visits/Hours: 3 (PICC line complications/labs)

Vitals Normal Range
BP Systolic: 90 - 160
BP Diastolic: 56 - 100
Pulse: 56 - 100
Respiratory: 12 - 28
Temperature: 96.5 - 100.5
O2 Saturation: 90 - 100

Vitals Physician Alert Range
Temperature: < 96.5 or > 100.5
Pulse: < 56 or > 100

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Respiratory: < 12 or > 28
BP Systolic: < 90 or > 160
BP Diastolic: < 56 or > 100
O2 Saturation: < 90
Weight: < 143 or > 153
Pain: > 7

SN

BEST PRACTICE FALL PREVENTION:

- Clinician will perform fall assessment and instruct patient /caregiver on fall prevention.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION:

- Clinician to perform drug regimen review/assessment to assess for potential clinically significant medications issues.
- Clinician to report potentially clinically significant medications issues to physician within 1 calendar day.
- Clinician to assess/instruct on high risk and other medications to include effects, potential adverse effects, interactions and duplicate therapy, dose route and frequency.

BEST PRACTICE MONITOR AND MITIGATE PAIN:

- Clinician will instruct patient /caregiver on methods to mitigate pain to include medications and non-pharmacological interventions to decrease pain.

LABS:

- Clinician to obtain CBC, CMP, CRP lab(s) via PICC line weekly. Flush line with 10ml NS, aspirate 20ml for waste and collect blood draw.

report results to physician: WIDC, Charlotte Ellenbogen

HOSPITALIZATION RISK FACTORS:

- Clinician will instruct patient/caregiver on ways to prevent unintentional weight loss and poor nutrition (high calorie foods, recording (wt/diet), etc
- Clinician will instruct patient and/or caregivers on polypharmacy, duplicate medications, adverse effects, et and compliance/understanding of medication regimen.
- Clinician will assess and teach patient and/or caregivers regarding declines in mental, emotional, or behavior status and when to report to the MD.
- Clinician to instruct patient and/or caregivers on ways to avoid exhaustion and fatigue to include adequate sleep, nutrition, exercise, etc.
- Clinician will instruct patient and/or caregivers on fall prevention to include clear pathways, adequate lighting, assistive devices, slow change in position, etc.

IV THERAPY:

- SN to instruct patient/caregiver on proper antibiotic infusion and flushes using the SASH method via PICC.
- SN to instruct patient/caregiver on signs and symptoms of infection and infiltration of IV/PICC site.
- SN to perform PICC dressing change weekly and prn dressing dislodgement/soiling as follows using sterile technique: Remove dressing pulling up towards insertion site. Open and remove securement device. Cleanse insertion site 4x4 in area with chlorhexidine using a side-to-side scrubbing motion for 30-60 seconds. Allow to dry completely. Apply skin protectant to peripheral IV insertion area, apply sterile securement device (state lock) to secure catheter, apply biotpatch to insertion site

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and cover with transparent dressing. Change caps.

• SN may remove PICC following completion of antibiotics.

Assessment Summary

Patient is a 46-year-old male with a history of diverticulosis and prior partial colectomy, who was recently admitted for abdominal pain and dysuria, found to have acute on chronic diverticulitis complicated by colovesical fistula and abdominal wall abscess. Colorectal surgeon recommends 6-8 weeks of optimization before surgery. Patient discharged on IV Zosyn for management of diverticulitis and abscesses.
Patient admitted to Home Health for Skilled Nurse to provide teaching and management of PICC line with weekly lab draws Patient alert and oriented x4 with pleasant affect. Patient is ambulatory without use of assistive device. He was prior a healthy and strong man before losing 30lbs. Lung sounds are clear in all lobes. He experiences SOB when climbing stairs or exerting strenuous activity. CV WNL. Patient reports regular urination. He reports constipation, cramping, abdominal pain, and gas at this time. He is managing abdominal pain with PRN tylenol and oxycodone ; he is trying to be conservative with oxy use to prevent further constipation. Discussed diet changes to promote more regular bowel movements and maintaining hydration. Patient does not have any wound. RUA PICC line in place with bruising noted at insertion site. Otherwise, PICC WNL. Dressing due to be changed on 2/27. Patient was taught how to switch IV abx balls while in hospital, and was able to perform switch of abx last night independently. IV abx due to be changed every 24 hours. Reinforced SASH method teaching with patient and he demonstrates understanding. This nurse obtained ordered weekly labs via PICC line, patient tolerated well and line functioning appropriately. Patient will require nursing visit on Friday for PICC line dressing change. Then visits will be done weekly for PICC line dressing change and weekly labs.
POC approved by Charlotte Ellenbogen, RN frequency 2w1, 1w8
Patient agreeable to POC

Frequency may be completed with both face to face visits and/or telehealth (if appropriate to meet patient treatment needs).
All qualified disciplines may do non-billable visits to supervise staff and collect oasis data as needed.
Within 5 days of the end of episode, a qualified professional may have a PRN visit to complete a comprehensive/OASIS assessment to establish the need for continuing home health services or discharge.
May accept orders from all physicians caring for the patient.
Any qualified disciplines may perform visit for resumption of care when patient discharged home from inpatient facility.
May resume all care prior to the hospitalization.
PAM Health at Home may discharge patient if remains in facility when certification period expires.
All skilled disciplines may check oxygen saturation prn sob.

I certify that the patient is confined to his/her home and needs intermittent skilled nursing, physical therapy, occupational therapy and/or speech therapy for the next 60 days.
I attest to the following:
1. VIGIL, JAMES P is under my care.
2. I have received the following patient records from PAM Health at Home:
a. Patient Summary Form
b. Hospital/LTC stay information (if applicable)
c. Home Health Plan of Care
d. And the signature below is my signature
e. These patient records were considered in advance of my certification of the patient's eligibility for home health services under Medicare.

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8. Date of Birth: 04/03/1979 **9. Sex:** ☒ M ☐ F

I certify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy for the next 60 days. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter has occurred on 02.19.2026 BY CHARLOTTE LOUISE ELLENBOGEN DO and it is related to the primary reason the patient requires home health services. PAM HEALTH AT HOME, HOME HEALTH AGENCY, MAY PERFORM SOC ON 02.23.2026.

Risk of Hospitalization

Risks: Unintentional weight loss of a total of 10 pounds or more in past 12 months; Multiple hospitalizations (2 or more) in the past 6 months; Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months; Currently reports exhaustion;
Interventions: Referrals - SN; Immunizations - Influenza; Medication Management - Reconciliation, Assess patients: Knowledge,Ability,Resources & Adherence, Education; Patient/Family Education; Fall Prevention Program; Individualized Patient Emergency Care Plan; Care Coordination (Physicians, Hospitals, Nursing homes...)
Risk Factors: Discharged from Hospital or Skilled Nursing Facility (M1000); Help with managing Medications needed (M2020); More than 2 Secondary Diagnoses (M1022 & M1024); ADL Assistance Needed (M2102 & M2110)

Psychosocial Assessment

No problem found

Cognitive Status

Cognitive status: No behaviors demonstrated

Advance Directive

Education needed and provided; Patient/Caregiver reports no Advanced Directives in place
Patient does not have an advance directive;
Have an Advanced Care Plan and it is documented in the medical record: No - Did not wish to provide an ADC or name a SDM;
Have a Surrogate Decision Maker and information documented in the medical record: No - Did not wish to provide an ADC or name a SDM;

Homebound Status

Patient Homebound Reasons
- Is the Patient Homebound: Yes
- Homebound Reasons: Residual weakness; Unable to safely leave home unassisted;
- Patient Chronic and Stable: Yes
- Patient leaving the home require Considerable and Taxing effort : Yes
- Comments:

22. Rehabilitation Potential/Discharge Plans/Goals :

Rehab Potential

Fair to achieve stated goals with skilled interventions and patient's compliance with plan of care.

Discharge Plan

to home under supervision of MD when goals met

Goals

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SN

GENERAL: DISEASE PROCESSES: - STG
- STG-Patient/Caregiver will be able to recall/demonstrate S/S of the following disease processes Acute Diverticulitis, Colovesical Fistula, IntraAbdominal Abscess for self management of disease process by EOE

BEST PRACTICE FALL PREVENTION: - LTG
- LTG-Patient/caregiver will verbalize understanding of interventions to prevent falls by end of episode.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION: - LTG
- LTG-Patient /caregiver will be able to name s/s of adverse side effects to report to MD by end of episode.
- LTG-Patient /caregiver will be able to verbalize/demonstrate correct dose, route, and frequency of all medications and will be able to recall adverse effects to report to MD by end of episode.

BEST PRACTICE MONITOR AND MITIGATE PAIN: - LTG
- LTG-Patient/caregiver will demonstrate decreased pain with ambulation as evidenced by increased activity tolerance by end of episode.

LABS: - STG
- STG-Patient /caregiver will demonstrate no complications from obtained lab(s) when performed. Results will be reported to physician as evidenced by clinical documentation.

HOSPITALIZATION RISK FACTORS: - LTG
- Patient and/or caregivers will verbalize understanding of ways to mitigate risk of return to the hospital and unplanned ER use.

IV THERAPY: - LTG
- Patient will remain free of complications with their vascular access device throughout the certification period.

IV THERAPY: - STG
- Patient/caregiver will provide good return demonstration regarding IV medication administration and vascular access device care (PICC)

23. Clinician's Signature and Date of Verbal SOC Where Applicable: (Electronically Signed)
Verbal SOC Date: 02/23/2026
Signed By: Dreyer, Alexis - RN on 02/23/2026 11:28 AM, Cosigned by Strait, Donna - RN

25. Date HHA Received Signed POT:

24. Physician's Name and Address
CHARLOTTE ELLENBOGEN MD
3885 Upham St, SUITE 200
Wheat Ridge, CO 80033,NPI: 1417914540, License#: 53322,Phone: (303) 425-9245, Fax: (303) 425-1378

26. I certify/ recertify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter occurred (or will occur) within timeframe requirements and it is related to the primary reason the patient requires home health service

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27. Attending Physician's Signature and Date Signed:

3.3.26

28. Anyone who misrepresents,falsifies, or conceals essential information required for payment of Federal funds may be subject to fine, imprisonment, or civil penality under applicable Federal laws.